

Attention-Deficit Hyperactivity Disorder, Adult - Self-Care

MCG Health
Chronic Care
28th Edition

CCG: C-0071 (CCG)

[Link to Codes](#)

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Assessment

Initial Consideration

Q1. Patient: What would you like to talk about today? (Check all that apply.) **QM**

- Emergency or urgent problems (eg, social determinants of health, intimate partner violence, suicidal ideation)^[A] Social Determinants of Health Screening Assessment [CCG](#) Intimate Partner Violence [CCG](#) Suicide or Personal Harm Risk [CCG](#)
- Condition information
- Provider coordination (eg, next appointment, treatment clarification) Appointment Scheduling [CCG](#)
- Medications
- Complications
- Monitoring and management
- Other health problems (eg, comorbid conditions, new symptoms)
- Other, specify: _____
- Nothing
- Unknown

Q2. Patient: Do you have any changes since the last time we talked? (Check all that apply.) **QM**

- Adhering to treatment or medication problems Readiness to Change [CCG](#) Medication Adherence [CCG](#)
- Administering medication Home Infusion Management [CCG](#) Injectable Medications [CCG](#)
- Affording medicine or supplies Financial Status and Benefits [CCG](#)
- Aspiration Aspiration Risk Management [CCG](#)
- Behavioral health provider appointment, specify: _____ Follow-Up - Provider Appointment [CCG](#)
- Blood pressure care Blood Pressure Monitoring [CCG](#)
- Blood sugar care Blood Glucose Monitoring [CCG](#) HbA1c Monitoring [CCG](#)
- Blood thinner care Anticoagulation Management [CCG](#)
- Bowel care Bowel Management [CCG](#)
- Bowel changes Bowel Symptoms [CCG](#)
- Caregiver Caregiver Resources Evaluation [CCG](#)
- Chemotherapy Chemotherapy [CCG](#)
- Cognitive impairment Cognitive Impairment [CCG](#) Behavioral Health and Cognition Evaluation with Caregiver [CCG](#)

- Communication or speech problems [Speech and Communication Problems](#)  CCG
- Community resource [Community Resources](#)  CCG
- Dehydration care [Dehydration Monitoring and Management](#)  CCG
- Depression symptoms [Depression Screening, Adult](#)  CCG
- Dialysis care [Dialysis Management](#)  CCG
- Doctor or specialist outpatient appointment, specify: _____ [Follow-Up - Provider Appointment](#)  CCG
- Energy level changes [Energy Level Changes](#)  CCG
- Enteral nutrition care [Enteral Nutrition Management](#)  CCG
- Exercising and staying active [Exercise](#)  CCG
- Fatigue or weakness [Fatigue or Weakness](#)  CCG
- Fluid restriction [Fluid Restriction](#)  CCG
- Functional status changes [Activities of Daily Living](#)  CCG [Instrumental Activities of Daily Living](#)  CCG
- Home infusion [Home Infusion Management](#)  CCG
- Hospital or emergency room visit, specify: _____ [Hospitalization Risk Assessment](#) 
 [CCG Post-Hospitalization Follow-Up](#)  CCG
- Medication changes [Medication Changes](#)  CCG
- Nebulizer care [Nebulizer Management](#)  CCG
- Neurologic function changes [Neurologic Function Changes](#)  CCG
- Ostomy care [Ostomy Management](#)  CCG
- Oxygen care [Oxygen Management](#)  CCG
- Pain, chronic [Chronic Pain](#)  CCG
- Pain, new or general [Pain Assessment](#)  CCG
- Parenteral nutrition care [Parenteral Nutrition Management](#)  CCG
- Peak flow meter care [Peak Flow Meter Use](#)  CCG
- Peripheral vascular (eg, lower leg redness or swelling) [Peripheral Vascular Problems](#)  CCG
- Physical impairments [Physical Impairments](#)  CCG
- Planning for long-term care needs [Advance Care Planning](#)  CCG
- Procedure, specify: _____ [Follow-Up - Provider Appointment](#)  CCG
- Pulmonary treatments [Pulmonary Treatments](#)  CCG
- Rehabilitation therapy, specify: _____ [Follow-Up - Provider Appointment](#)  CCG
- Respiratory changes [Respiratory Symptoms](#)  CCG
- Safety [Safety](#)  CCG
- Scheduling care [Appointment Scheduling](#)  CCG
- Sleeping problems [Sleep](#)  CCG
- Social support [Social Support](#)  CCG
- Stasis ulcer care [Stasis Ulcer Management](#)  CCG
- Testing (eg, laboratory, imaging), specify: _____ [Follow-Up - Provider Appointment](#) 
 CCG
- Tracheostomy care [Tracheostomy Management](#)  CCG
- Transportation [Transportation](#)  CCG
- Understanding healthcare needs [Health Literacy](#)  CCG
- Urinary catheter care [Urinary Catheter Management](#)  CCG
- Urinary changes [Urinary Symptoms](#)  CCG
- Venous thromboembolism symptoms [Venous Thromboembolism](#)  CCG
- Ventilator care [Ventilator Management](#)  CCG
- Weight gain [Nutritional Status, Adult](#)  CCG [Nutritional Status, Pediatric](#)  CCG [Nutritional Status, Infant](#)  CCG
- Weight loss [Nutritional Status, Adult](#)  CCG [Nutritional Status, Pediatric](#)  CCG [Nutritional Status, Infant](#)  CCG

- Wound care Wound Management  CCG
- Other, specify: _____
- No
- Unknown Hospitalization Risk Assessment  CCG

Condition Knowledge

Q1. Patient: What causes attention-deficit hyperactivity disorder (ADHD)? (Check items understood.)

- Actual cause unknown
- Changes in brain during childhood
- Family history (genetics)
- Other, specify: _____
- Unknown [P1]
- Nothing [P1]
- Not all appropriate information known [P1]

Q2. Case Manager: Rate the patient's understanding of the causes of ADHD. **QM**

- 90% or more
- 75% to 89% [P1]
- 50% to 74% [P1]
- Less than 50% [P1]
- Unknown [P1]

Q3. Patient: What are the symptoms of ADHD? (Check items understood.)

- Difficulty with organization
- Difficulty with solving problems
- Doing things without thinking first (impulsive)
- Easily frustrated
- Problems with close attention to detail
- Trouble coping with stress
- Trouble following complex instructions
- Trouble getting along with people at home, in school, or at work
- Trouble keeping a job
- Trouble paying attention
- Other, specify: _____
- Unknown [P1]
- Nothing [P1]
- Not all appropriate information known [P1]

Q4. Case Manager: Rate the patient's understanding of the symptoms of ADHD. **QM**

- 90% or more
- 75% to 89% [P1]
- 50% to 74% [P1]
- Less than 50% [P1]
- Unknown [P1]

Q5. Patient: How can you care for your ADHD to keep it from getting worse and causing problems? (Check items understood.) **QM**

- Adhere to self-care plan
- Ask for help when feelings are overwhelming
- Break complex projects into smaller tasks
- Exercise
- Follow an eating (diet) plan
- Keep a diary about feelings, activities, eating times, and medicines taken
- Keep all doctor appointments
- Manage situation or event triggers
- Manage stress
- Organize work space at home and at work
- Seek treatment for anxiety or mood disorders
- Set realistic expectations for feeling better, managing symptoms, and recovery
- Set small goals every day
- Stop using alcohol, tobacco, or drugs
- Take medicines as ordered
- Other, specify: _____

- Unknown [P1]
- Nothing [P1]
- Not all appropriate information known [P1]

Q6. Case Manager: Rate the patient's understanding of how to care for ADHD. **QM**

- 90% or more
- 75% to 89% [P1] [P2]
- 50% to 74% [P1] [P2]
- Less than 50% [P1] [P2]
- Unknown [P1] [P2]

Q7. Patient: What activities would you be able to do if you didn't have problems with your ADHD? (Check all that apply.) **QM**

- Activities of daily living (ADL) without assistance [P2]
- Cook [P2]
- Drive [P2]
- Exercise [P2]
- Go out with friends [P2]
- Go to events [P2]
- Hobbies [P2]
- Play sports [P2]
- Play with children or grandchildren [P2]
- Shop [P2]
- Travel [P2]
- Work [P2]
- Other, specify: _____ [P2]
- Unknown [P1]

Provider Information

Q1. Patient: What doctor manages your ADHD? **QM**

- Psychiatrist or psychologist, name and phone number: _____ [Q2]
- Behavioral health therapist, name and phone number: _____ [Q3]
- Primary doctor, name and phone number: _____
- Other specialist, name and phone number: _____ [Q5]
- Don't have doctor [P1]
- Unknown [P1]

Q2. Patient: When was your last visit with the psychiatrist or psychologist?

- Less than 1 month ago
- 1 to 3 months ago [P2]
- 4 to 6 months ago [P2]
- More than 6 months ago [P2]
- Unknown [P2]
- Never [P2]

Q3. Patient: When was your last visit with a behavioral health therapist?

- Less than 1 month ago
- 1 to 3 months ago [P2]
- 4 to 6 months ago [P2]
- More than 6 months ago [P2]
- Unknown [P2]
- Never [P2]
- Not applicable

Q4. Patient: When was your last visit with your primary doctor? **QM**

- Within the past 3 months
- 4 to 6 months ago [P2]
- More than 6 months ago [P2]
- Unknown [P2]
- Never [P2]

Q5. Patient: When was your last visit with the specialist?

- Within the past 3 months
- 4 to 6 months ago [P2]
- More than 6 months ago [P2]
- Unknown [P2]
- Never [P2]

Q6. Patient: What other specialists have you seen within the last 12 months? (Check all that apply.) **QM**

- Behavioral health provider
- Cardiologist
- Dentist
- Dietitian
- Endocrinologist
- Gastroenterologist
- General surgeon
- Genetic counselor
- Geriatrician
- Gynecologist
- Hematologist
- Hepatologist
- HIV/AIDS
- Infectious disease
- Nephrologist
- Neurologist
- Neurosurgeon
- Obstetrician
- Oncologist
- Ophthalmologist
- Orthopedic surgeon
- Orthopedist
- Otolaryngologist
- Pain management
- Physiatrist
- Plastic surgeon
- Podiatrist
- Pulmonologist
- Rheumatologist
- Transplant specialist
- Urologist
- Other, specify: _____
- None [P1]
- Unknown [P1]

Q7. Patient: Do you have a written self-care plan from your doctor on how to care for ADHD?[B] **QM**

- Yes [Q8]
- No [P1] [P2]
- Unknown [P1] [P2]

Q8. Patient: What are the components of your self-care plan for ADHD? (Check all that apply.) **QM**

- Exercise plan
- Knowing what medicines you take and why and how to take them
- Managing side effects of medicines
- Medicine use
- Nutrition plan
- Recognizing the symptoms of gradual worsening or problems with the condition
- Regular follow-up appointments for wellness checks
- When to call the doctor and when to get emergency help
- Other, specify: _____ [P2]
- Unknown [P2]
- Not applicable
- Not all appropriate information known [P2]

Q9. Case Manager: Is the patient able to understand and adhere to the self-care plan? **QM**

- Yes, independently
- Yes, with occasional reminders or caregiver support
- Yes, with frequent reminders or caregiver support
- No [P3]
- Unknown [P3]

Medication Information**Q1. Patient:** Has any medicine for ADHD been prescribed by your doctor? If so, is it taken as ordered? **QM**

- Yes, and taken as ordered [Q2]
- Yes, but not taken as ordered [Q2] [P4] Medication Adherence [CCG](#)
- No medicine prescribed [P4]
- Unknown [P4]

Q2. Patient: What medicines do you take for ADHD? (Check all that apply.) **QM**

- Atomoxetine [Q3] [P5] [P6]
- Stimulants (eg, dextroamphetamine, lisdexamfetamine, methylphenidate) [Q3] [P5] [P7]
- Other, specify: _____ [Q3] [P4] [P5]
- Not applicable [P4]
- Unknown [P5]

Q3. Patient: Are you having side effects from the ADHD medicines? (Check all that apply.)

- Confusion [P5]
- Difficulty falling asleep or sleeping too much [P5]
- Fast, pounding heartbeat [P5]
- Hallucinations [P5]
- Headache [P5]
- Increase in blood pressure [P5]
- Involuntary movements [P5]
- Irritability [P5]
- Loss of appetite [P5]
- Upset stomach (nausea) [P5]
- Weight loss [P5]
- Other, specify: _____ [P5]
- None
- Unknown [P5]
- Not applicable

Q4. Case Manager: Rate the patient's understanding of the purpose and side effects of the medications for ADHD. **QM**

- 90% or more
- 75% to 89% [P1] [P5]
- 50% to 74% [P1] [P5]
- Less than 50% [P1] [P5]
- Unknown [P1] [P5]

Monitoring and Management**Q1. Patient:** What problems with ADHD have you had in the past month? (Check all that apply.)

- Difficulty solving problems [P2] [P3]
- Difficulty with organization [P2] [P3]
- Doing things without thinking first (impulsive) [P2] [P3]
- Easily bored [P2] [P3]
- Easily frustrated [P2] [P3]
- Losing things [P2] [P3]
- Procrastinating [P2] [P3]
- Trouble coping with stress [P2] [P3]
- Trouble following instructions [P2] [P3]
- Trouble getting along with people at home, in school, or at work [P2] [P3]
- Trouble keeping a job [P2] [P3]
- Trouble paying attention [P2] [P3]
- Other, specify: _____ [P2] [P3]

- None
- Unknown [P2] [P3]

Q2. Patient: Do you use tobacco, alcohol, or drugs? (Check all that apply.) **QM**

- Yes, tobacco use Tobacco Use [CCG](#)
- Yes, alcohol use Alcohol Use [CCG](#)
- Yes, drug use Substance Use [CCG](#)
- No
- Unknown Tobacco Use [CCG](#) Alcohol Use [CCG](#) Substance Use [CCG](#)

Q3. Patient: How much do you weigh and how tall are you? **QM**

- Weight: _____
- Height: _____
- Unknown [P8]

Q4. Case Manager: What is the patient's body mass index (BMI)? BMI Calculator

- Less than 18.5 [P8] [P9] Nutritional Management to Gain Weight [CCG](#) Body Weight Planner [CCG](#)
- 18.5 to 24.9
- 25 to 29.9 [P8] [P9] Nutritional Management to Lose Weight [CCG](#) Body Weight Planner [CCG](#)
- 30 or greater [P8] [P9] Nutritional Management to Lose Weight [CCG](#) Body Weight Planner [CCG](#)
- Unknown [P8]

Q5. Patient: Do you know what your body weight should be?

- Yes, it should be: _____
- No [P8]
- Unknown [P8]

Q6. Patient: What do you eat and drink in a typical day? (Check all reported.)

- Alcohol, specify types and number of daily servings: _____ [Q7] [Q8] [Q9] [Q10] [Q11]
- Dairy foods, specify types and number of daily servings: _____ [Q7] [Q8] [Q9] [Q10] [Q11]
- Fats, specify types and number of daily servings: _____ [Q7] [Q8] [Q9] [Q10] [Q11]
- Fruits, specify types and number of daily servings: _____ [Q7] [Q8] [Q9] [Q10] [Q11]
- Fruit juices, specify types and number of daily servings: _____ [Q7] [Q8] [Q9] [Q10] [Q11]
- Grains, specify types and number of daily servings: _____ [Q7] [Q8] [Q9] [Q10] [Q11]
- Meats (beef, chicken, fish), specify types and number of daily servings: _____ [Q7] [Q8] [Q9] [Q10] [Q11]
- Milk, specify types and number of daily servings: _____ [Q7] [Q8] [Q9] [Q10] [Q11]
- Sodas, specify types and number of daily servings: _____ [Q7] [Q8] [Q9] [Q10] [Q11]
- Supplements, specify types and number of daily servings: _____ [Q7] [Q8] [Q9] [Q10] [Q11]
- Sweets, specify types and number of daily servings: _____ [Q7] [Q8] [Q9] [Q10] [Q11]
- Vegetables, specify types and number of daily servings: _____ [Q7] [Q8] [Q9] [Q10] [Q11]
- Water, specify number of daily servings: _____ [Q7] [Q8] [Q9] [Q10] [Q11]
- Other, specify: _____ [Q7] [Q8] [Q9] [Q10] [Q11] [P8]
- No food by mouth [Q11] [P8] [P9]
- Unknown [Q7] [Q8] [Q9] [Q10] [Q11] [P8] [P9]

Q7. Patient: How often do you eat each day? (Check all reported.) **QM**

- 3 meals or more [P8] [P9]
- 2 meals
- 1 meal [P8] [P9]
- 0 meals [P8] [P9]
- 2 snacks or more [P8] [P9]
- 1 snack
- 0 snacks
- Other, specify: _____ [P8] [P9]
- Not applicable
- Unknown [P8] [P9]

Q8. Patient: How much fluid (such as water, juice, coffee, tea, milk) do you have per day? **QM**

- Less than 3 cups (720 mL) [P8] [P9]

- 3 to 5 cups (720 mL to 1.2 L) [P8] [P9]
- More than 5 cups (1.2 L)
- Unknown [P8] [P9]

Q9. Patient: Are you trying to gain or lose weight?

- Trying to gain weight [P8] [Nutritional Management to Gain Weight](#) [CCG](#) [Body Weight Planner](#) [CCG](#)
- Trying to lose weight [P8] [Nutritional Management to Lose Weight](#) [CCG](#) [Body Weight Planner](#) [CCG](#)
- Neither
- Unknown [P8]

Q10. Patient: Does your doctor want you to follow a special diet program? (Check all that apply.) **QM**

- Yes, DASH (low sodium) [P8] [Diet Knowledge Insufficient - DASH Diet](#) [CCG](#)
- Yes, heart healthy [P8] [Diet Knowledge Insufficient - Heart Healthy](#) [CCG](#)
- Yes, kidney diet [P8] [Diet Knowledge Insufficient - Renal Dysfunction](#) [CCG](#)
- Yes, low carb or low glycemic index [P8] [Diet Knowledge Insufficient - Low Glycemic Index](#) [CCG](#)
- Yes, pregnancy diet [P8] [Diet Knowledge Insufficient - Pregnancy](#) [CCG](#)
- Yes, other: _____ [P8]
- No [P8]
- Unknown [P8]

Q11. Patient: Do you get nutritional treatments or have an altered diet? If yes, why? (Check all that apply.) **QM**

- Behavioral health provider seen, specify: _____ [P9]
- Dental soft diet, specify: _____ [P9]
- Diet supplements, specify: _____ [P9]
- Dietitian seen or following, specify: _____ [P9]
- Enteral nutrition (tube feeding), specify: _____ [P9] [Enteral Nutrition Management](#) [CCG](#)
- IV fluids (hydration), specify: _____ [P9]
- Liquid diet, specify: _____ [P9]
- Mechanical soft diet, specify: _____ [P9]
- Parenteral nutrition, specify: _____ [P9] [Parenteral Nutrition Management](#) [CCG](#)
- Procedure(s) done by a doctor in office, specify: _____ [P9]
- Pureed diet, specify: _____ [P9]
- Speech-language pathology (SLP), specify: _____ [P9]
- Supervised during nutritional intake, specify: _____ [P9]
- Surgery, specify: _____ [P9]
- Thickened liquid diet, specify: _____ [P9]
- Other, specify: _____ [P9]
- No
- Unknown [P8]

Q12. Patient: Do you know how to read food labels?

- Yes
- No [P10]
- Unknown [P10]

Q13. Patient: Do you get regular exercise? **QM**

- Yes
- No [P2]
- Unknown [P2]

Q14. Patient: Has your mood changed over the past 2 weeks?(1)(2) [PHQ-2 Calculator](#) **QM**

- Yes [Q15]
- No
- Unknown

Q15. Patient: What changes in your mood have you had? (Check all that apply.) **QM**

- Change in usual activity (eg, working excessively, no longer talking to friends, or going on shopping sprees) [Depression Screening, Adult](#) [CCG](#)

- Down, hopeless, or depressed Depression Screening, Adult [CCG](#)
- Forgetting a lot or getting angry or frustrated Depression Screening, Adult [CCG](#)
- Little interest or pleasure in doing things Depression Screening, Adult [CCG](#)
- Quick mood changes Depression Screening, Adult [CCG](#)
- Other, specify: _____ Depression Screening, Adult [CCG](#)
- Not applicable

Q16. Patient: Have you participated in any programs related to ADHD? (Check all that apply.)

- Behavioral health counseling
- Community education program
- Exercise program
- Support group
- Not now, but in the past [Q17] [P2] [P3]
- Other, specify: _____
- None [P2] [P3]
- Unknown [P2] [P3]

Q17. Patient: When was the last time you participated in a therapy program?

- Within the past 3 months
- 4 to 11 months ago
- 12 months ago or more
- Other, specify: _____ [P2]
- Unknown [P2]
- Not applicable

Q18. Patient: Are you having any problems with your ADHD self-care needs? (Check all that apply.) **QM**

- Affording medicine or supplies [P2] [P3] Financial Status and Benefits [CCG](#)
- Blood pressure care [P2] [P3] Blood Pressure Monitoring [CCG](#)
- Blood sugar care [P2] [P3] Blood Glucose Monitoring [CCG](#)
- Caregiver help needed [P2] [P3] Caregiver Resources Evaluation [CCG](#)
- Community resource information needed [P2] [P3] Community Resources [CCG](#)
- Daily care of child [P2] [P3] Caregiver Resources Evaluation [CCG](#)
- Exercising and staying active [P2] [P3] Exercise [CCG](#)
- Safety in the home [P2] [P3] Safety [CCG](#)
- Sleeping problems [P2] [P3] Sleep [CCG](#)
- Social support [P2] [P3] Social Support [CCG](#)
- Taking medicine as prescribed [P2] [P3] Medication Adherence [CCG](#)
- Transportation for care [P2] [P3] Transportation [CCG](#)
- Trouble gaining weight [P2] [P3] Nutritional Status, Adult [CCG](#) Nutritional Management to Gain Weight [CCG](#)
- Trouble losing weight [P2] [P3] Nutritional Status, Adult [CCG](#) Nutritional Management to Lose Weight [CCG](#)
- Trouble making lifestyle changes [P2] [P3] Readiness to Change [CCG](#)
- Trouble understanding how to do self-care [P2] [P3] Health Literacy [CCG](#)
- Other, specify: _____ [P2] [P3]
- No
- Unknown [P2] [P3]

Q19. Case Manager: Rate the patient's understanding of the monitoring and management of ADHD. **QM**

- 90% or more
- 75% to 89% [P1] [P2]
- 50% to 74% [P1] [P2]
- Less than 50% [P1] [P2]
- Unknown [P1] [P2]

Other Health Considerations

Q1. Patient: What vaccinations have you had? (Check all that apply.) **QM**

- Coronavirus disease 2019 (COVID-19) [Q2]

- *Haemophilus influenzae* type b (Hib) [Q2] [Q3]
- Hepatitis A [Q2]
- Hepatitis B [Q2]
- Human papillomavirus (HPV) [Q2]
- Influenza [Q2] [Q3]
- Measles, mumps, rubella (MMR) [Q2]
- Meningococcal 4-valent conjugate (MenACWY) or polysaccharide (MPSV4) [Q2]
- Meningococcal B (MenB) [Q2]
- Pneumococcal 13-valent conjugate (PCV13) [Q2] [Q4]
- Pneumococcal 15-valent conjugate (PCV15) [Q2] [Q4]
- Pneumococcal 20-valent conjugate (PCV20) [Q2] [Q4]
- Pneumococcal 23-valent polysaccharide (PPSV23) [Q2] [Q4]
- Polio (inactivated) [Q2]
- Respiratory syncytial virus (RSV) [Q2]
- Rotavirus [Q2]
- Tetanus booster [Q2]
- Tetanus, diphtheria, pertussis (Td/Tdap) [Q2]
- Varicella [Q2]
- Zoster [Q2]
- Other, specify: _____ [Q2]
- None, because: _____ [P11]
- Unknown [P11]

Q2. Patient: Are you up to date with the vaccinations you need (for age, booster status, or medical condition)? **QM**

- Yes
- No, because: _____ [Q5] [P11]
- Unknown [Q5] [P11]

Q3. Patient: When was your last flu shot? **QM**

- In the past year
- More than a year ago [Q5] [P11]
- Never [Q5] [P11]
- Unknown [Q5] [P11]

Q4. Patient: When was your last pneumonia shot?

- In the last year
- 2 to 4 years ago
- 4 to 5 years ago [Q5] [P11]
- More than 5 years ago [Q5] [P11]
- Never [Q5] [P11]
- Unknown [Q5]

Q5. Patient: Do you have an appointment scheduled to get caught up on your vaccinations?

- Yes
- No [P11]
- Unknown [P11]

Q6. Patient: Is there someone you talk to about your problems?

- Yes, name and relationship: _____
- No [B1] [P12]
- Unknown [B1] [P12]

Q7. Patient: Do you have someone to count on when you need help?

- Yes, name and relationship: _____
- No [B1] [P12]
- Unknown [B1] [P12]

Q8. Patient: Is there someone who can help you feel better when you are stressed, sad, or upset?

- Yes, name and relationship: _____
- No [B1] [P12]
- Unknown [B1] [P12]

Q9. Patient: Is there someone who accepts you for who you are, the good and bad?

- Yes, name and relationship: _____
- No [B1] [P12]
- Unknown [B1] [P12]

Q10. Patient: Is there someone you can count on to care about you no matter what is happening?

- Yes, name and relationship: _____
- No [B1] [P12]
- Unknown [B1] [P12]

Q11. Patient: Does your social support help you as much as you need? **QM**

- Yes
- No [B1] [P12]
- Unknown [B1] [P12]

Q12. Patient: Do you have other health problems or conditions that might make ADHD worse? (Check all that apply.) **QM**

- Alcohol or substance misuse [P2] [P3] Substance-Related Disorders - Self-Care [↗](#) CCG Substance Use [↗](#) CCG
- Anxiety [P2] [P3] Anxiety - Self-Care [↗](#) CCG
- Autism spectrum disorder [P2] [P3]
- Bipolar disorder [P2] [P3] Bipolar Disorder - Self-Care [↗](#) CCG
- Depression [P2] [P3] Depression - Self-Care [↗](#) CCG
- Diabetes [P2] [P3] Diabetes, Adult - Self-Care [↗](#) CCG
- High blood pressure [P2] [P3] Hypertension - Self-Care [↗](#) CCG
- Obesity or overweight [P2] [P3] Obesity, Adult - Self-Care [↗](#) CCG
- Physical inactivity [P2] [P3] Exercise [↗](#) CCG
- Pregnancy [P2] [P3] High-Risk Pregnancy - Self-Care [↗](#) CCG
- Seizures [P2] [P3] Seizure Disorders - Self-Care [↗](#) CCG
- Thyroid disease [P2] [P3]
- Tourette syndrome [P2] [P3]
- Other, specify: _____ [P2] [P3]
- No
- Unknown [P1]

Patient Understanding Evaluation

Q1. Case Manager: Does the patient understand all the causes of ADHD?

- Yes
- No [P1]
- Unknown [P1]

Q2. Case Manager: Does the patient understand all the symptoms of ADHD?

- Yes
- No [P1]
- Unknown [P1]

Q3. Case Manager: Does the patient understand how to care for ADHD? **QM**

- Yes
- No [P1] [P2]
- Unknown [P1] [P2]

Q4. Case Manager: Does the patient know when and where to get help for problems?

- Yes
- No [P1] [P3]
- Unknown [P1] [P3]

Q5. Case Manager: Does the patient have the ability to understand and follow the self-care plan? **QM**

- Yes
- No [P3]
- Unknown [P3]

Q6. Case Manager: Does the patient understand the purpose and side effects of their ADHD medication?

- Yes
- No [P5]
- Unknown [P5]

Q7. Case Manager: Is the patient having trouble coping with their ADHD? **QM**

- Yes [P3]
- No
- Unknown [P3]

Q8. Case Manager: Does the patient agree to education? **QM**

- Yes
- No, explain why: _____ [P3] Readiness to Change  CCG
- Unknown [P3]

Quality Measures

- Disease Management Quality Measures:
 - Information provided from this question may be requested by various accrediting organizations.
- HEDIS Measures include(3):
 - Tobacco use is assessed and addressed.
 - Alcohol and substance misuse is assessed and addressed.

Plan of Care

Barriers

B1. Inadequate social support

Problems

P1. ADHD information needed

- **Goal:** ADHD diagnosis better understood(4)(5) 
- **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling
 - **EDUCATE: Definition:** Attention-deficit hyperactivity disorder (ADHD) is a developmental neurobiological disorder.(6)
 - It presents in childhood and can persist in varying degrees into adulthood, and is characterized by attention and/or hyperactivity and impulsivity problems.
 - Symptoms may change in duration and intensity over time.
 - **EDUCATE: Causes:** The cause of ADHD is not known. Factors that have been identified include biochemical influences and genetics.[C](6)
 - **EDUCATE: Symptoms(7):**
 - Behavioral changes (eg, sudden mood swings, aggression)
 - Concentration difficulty
 - Coping problems
 - Emotional affect alteration (eg, flat, inappropriate, blunt, labile)
 - Impulse control problems
 - Social problems (eg, unpopular, argumentative)
 - Thought process problems (eg, planning, reasoning, decision making)
 - **EDUCATE:** Secondary complications of ADHD(6):
 - Fewer close friends and trouble keeping those friendships
 - Tend to have a negative perception of marital and family life and have a higher incidence of separation and divorce
 - Problems with communication, roles, and problem solving
 - Lower self-esteem
 - Vocational challenges, such as frequent changes in employment (poor performance, quit or fired)
 - **EDUCATE: Diagnosis:** Based on medical history, risk factors, physical examination, and diagnostic tests(8)
 - **EDUCATE: Treatment:**
 - ADL and IADL support
 - Behavioral health therapy options
 - Comorbid condition management

- Self-care activities (eg, diet, activity, medication)
- Medication management
- No cure
- **EDUCATE: Treatment goals(6):**
 - Functional improvement and maintenance (eg, ADL, IADL)
 - Psychosocial functioning improved and maintained
 - Quality of life improvement(9)
 - Safety (for self and others)
 - Symptom frequency and severity reduced
- **EDUCATE: Resources:**
 - Attention Deficit Disorder Association: www.add.org
 - Children and Adults with Attention-Deficit/Hyperactivity Disorder (CHADD): www.chadd.org
 - National Suicide Prevention Lifeline: www.988lifeline.org
 - Smokefree. National Institutes of Health: www.smokefree.gov
- **COORDINATE:** Behavioral health services referral, if indicated
- **COORDINATE:** Psychiatrist referral, if indicated
- **COORDINATE:** Laboratory testing, if indicated(6)
- **COORDINATE:** Social worker referral, as needed(10)
- **COORDINATE:** Care team meeting, if needed(11)(12)
- **COORDINATE:** Communication between care team members and care plan reconciliation(11)(13)(14)
- **COORDINATE:** Follow-up contact to review ADHD understanding, referral status, and need for further assistance(15)(16)
- **SEND:** Your care plan - Attention-deficit hyperactivity disorder, adult  (Spanish)
- **SEND:** What you need to know - Attention-deficit hyperactivity disorder, adult  (Spanish)
- **SEND:** When to get help - Attention-deficit hyperactivity disorder, adult  (Spanish)
- **SEND:** Daily care for your condition - Attention-deficit hyperactivity disorder, adult  (Spanish)
- **SEND:** Be ready for your doctor visit - Attention-deficit hyperactivity disorder, adult  (Spanish)
- **SEND:** Be ready with questions for your doctor - Attention-deficit hyperactivity disorder, adult  (Spanish)
- **SEND:** Tips for good mental health  (Spanish)

P2. ADHD self-care needed

- **Goal:** ADHD self-care plan developed, implemented, and maintained(4)(17)(18) 
- **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling
 - **ASSIST:** Self-care plan creation between patient and provider (eg, goals, management, treating symptoms, getting help) [D]
 - **ASSIST:** Communication between multiple providers(19)
 - **EDUCATE: Self-care plan information(6):**
 - Adherence to plan necessary to achieve goals, and complications of nonadherence (eg, exacerbations, progression, comorbidities)
 - Behavioral health monitoring and care (eg, depression monitoring and management)
 - Comorbid condition management
 - Diet considerations (eg, reduce caffeine and alcohol, keep food diary)(20)
 - Exercise program within provider's recommendations(21) 
 - Functional changes care (eg, adaptations and support for ADL and IADL)(22)
 - Health checks, diary, and action plans (eg, reduce distractions, set goals, stick to a daily routine)
 - Medication purpose, use, effects, and considerations
 - Safety awareness and management (eg, burn prevention, fall prevention)
 - Sleep hygiene (eg, regular pattern, calm sleep environment)
 - Stress management (eg, relaxation and coping techniques)
 - Symptom management
 - Tobacco cessation plan
 - **EDUCATE: When to get emergency help(23):**
 - Suicidal or homicidal actions or plans(24)
 - Crisis plan not working
 - Hallucinations or delusions, increased or new

- Psychotic behavior (eg, threatening)
- Heart attack (eg, persistent chest pain; pain radiating to the arms, shoulders, neck, back, and/or jaw; cool clammy skin)(25)
- Stroke (eg, face drooping, arm weakness, speech problems, bad headache, vision changes, loss of balance or coordination)(26)
- Respiratory impairment (eg, gasping for air, choking, cyanosis, chest pain, severe dyspnea)(27)
- Vision change, sudden
- Seizures, new or complicated (eg, seizure longer than 5 minutes, multiple seizures with loss of consciousness, head injury during seizure)(28)
- **EDUCATE: When to call the provider for behavior changes:**
 - Fatigue, unrelenting
 - Insomnia
 - Medication problems (eg, side effects, access, administration)
 - Mood instability (eg, extreme or rapid changes)
 - Self-care changes (eg, ADL or IADL problems)
 - Substance misuse (eg, increased use, use interferes with ADL/IADL)
 - Appointments missed (eg, support group, provider)
 - Cognitive changes (eg, thinking problems, forgetfulness worsens)
- **EDUCATE: When to call the provider(6):**
 - Blood pressure outside of provider's parameters
 - Emergency department or urgent care visit
 - Falls
 - Malnutrition (eg, weight loss, dehydration, muscle loss)
 - Medications newly ordered from other providers
 - Pregnant or planning to get pregnant
 - Weight gain or loss without reason
- **COORDINATE:** Behavioral health services referral, if indicated
- **COORDINATE:** Psychiatrist referral, if indicated
- **COORDINATE:** Community services referral, as needed
- **COORDINATE:** School or education support, as needed
- **COORDINATE:** Support group referral, as needed
- **COORDINATE:** Vocational rehabilitation referral, as needed
- **COORDINATE:** Tobacco use treatment, as needed
- **COORDINATE:** Care team meeting, if needed(11)(12)
- **COORDINATE:** Communication between care team members and care plan reconciliation(11)(13)(14)
- **COORDINATE:** Follow-up contact with patient to evaluate understanding and adherence to self-care plan, referral status, and need for further assistance(15)(16)
- **SEND:** Your care plan - Attention-deficit hyperactivity disorder, adult  (Spanish)
- **SEND:** What you need to know - Attention-deficit hyperactivity disorder, adult  (Spanish)
- **SEND:** When to get help - Attention-deficit hyperactivity disorder, adult  (Spanish)
- **SEND:** Daily care for your condition - Attention-deficit hyperactivity disorder, adult  (Spanish)
- **SEND:** Be ready for your doctor visit - Attention-deficit hyperactivity disorder, adult  (Spanish)
- **SEND:** Be ready with questions for your doctor - Attention-deficit hyperactivity disorder, adult  (Spanish)
- **SEND:** Choosing a place to live  (Spanish)
- **SEND:** Help with sleeping or insomnia  (Spanish)
- **SEND:** How to stop smoking  (Spanish)
- **SEND:** Tips for good mental health  (Spanish)
- **SEND:** Symptoms diary (Illustrated)  (Spanish)
- **Goal:** Behavioral health care initiated and maintained
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling
 - **ASSIST:** Identifying coping mechanisms
 - **EDUCATE:** Symptom management includes(6):
 - Daily care (eg, develop and maintain routine)(7)
 - Self-monitoring (eg, develop list of important tasks to do each day, and reference list frequently)

- Realistic goal setting. Break large tasks into smaller, easier-to-do steps.
- Distraction avoidance techniques (eg, turn off TV while doing homework, use headphones to block noises)
- Activity, regular pattern
- Sleep hygiene (eg, regular pattern, calm sleep environment)
- Trigger avoidance (eg, high-stress situations, sleep deprivation)
- Stress management (eg, relaxation and coping techniques)
- Crisis care plan: turn off distractions (eg, TV, radio), seek emergent care, call hotline, call police if escalates; caregivers advised to not argue
- Safety awareness and management (eg, remove suicide risks, de-escalation techniques)
- Self-care activities (eg, diet, activity, medication)
- Psychotherapy (eg, cognitive behavioral therapy (CBT), individual or group therapy, family therapy)^{[E](29)(30)}
- Appointment scheduling and attendance
- **COORDINATE:** Behavioral health services referral, if indicated
- **COORDINATE:** Specialist referral for comorbidities, if indicated
- **COORDINATE:** Treatment program referral, if indicated
- **COORDINATE:** Telehealth referral, if indicated^[F]
- **COORDINATE:** Care team meeting, if needed⁽¹¹⁾⁽¹²⁾
- **COORDINATE:** Communication between care team members and care plan reconciliation⁽¹¹⁾⁽¹³⁾⁽¹⁴⁾
- **COORDINATE:** Follow-up contact with patient to evaluate understanding and adherence to self-care plan, referral status, and need for further assistance⁽¹⁵⁾⁽¹⁶⁾
- **SEND:** Be ready for your doctor visit - Attention-deficit hyperactivity disorder, adult  (Spanish)
- **SEND:** Be ready with questions for your doctor - Attention-deficit hyperactivity disorder, adult  (Spanish)
- **SEND:** Be ready for your doctor visit - Depression  (Spanish)
- **SEND:** Be ready with questions for your doctor - Depression  (Spanish)
- **SEND:** Be ready for your doctor visit - Substance-related disorders  (Spanish)
- **SEND:** Be ready with questions for your doctor - Substance-related disorders  (Spanish)

P3. ADHD potential complications and comorbidities monitoring needed

- **Goal:** ADHD potential complication or comorbidity evaluated and addressed⁽⁴⁾
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling for evaluation of problems, comorbidities, and self-care plan^[G]:
 - Recently hospitalized patient's evaluation within 1 week of discharge^N
 - Recently hospitalized patient's evaluation with outpatient healthcare providers (eg, nurse, social worker) within 1 to 3 days of discharge⁽¹⁵⁾
 - Appointment every 3 to 6 months when patient is symptom free
 - Appointment once a month or more for patient with questionable adherence to treatment plan
 - **EDUCATE:** When and how to get help for potential worsening ADHD problems
 - **EDUCATE:** Treatment and monitoring for comorbid conditions (eg, substance misuse, anxiety, depression, obesity)⁽⁸⁾⁽⁹⁾
 - **EDUCATE:** Follow-up appointments for assessment of clinical status⁽³⁰⁾
 - **EDUCATE:** Stimulant medications prescribed for ADHD may worsen comorbid conditions⁽³¹⁾⁽³²⁾:
 - Heart disease or arteriosclerosis⁽⁸⁾
 - Increased tics in Tourette disorder
 - Hyperthyroidism
 - Hypertension
 - Taking monoamine oxidase inhibitors (MAOIs) concurrently
 - Glaucoma
 - Insulin dose changes may be needed. Monitor blood sugar 2 to 3 times per day or as instructed, and evaluate possible changes in food intake, weight, and activity.⁽³¹⁾
 - **EDUCATE:** Stimulant misuse signs and symptoms include⁽³³⁾⁽³⁴⁾:
 - Crushing, chewing, or taking with alcohol
 - Drug-seeking behaviors
 - Depression or mood changes
 - Anxiety or panic attacks
 - Paranoia or psychosis
 - Legal problems
 - Aggression
 - Tremors

- Seizures
- **EDUCATE:** Effects of behavioral health condition and conjunctive medication usage during pregnancy and lactation(35)
- **COORDINATE:** Behavioral health services referral, if indicated(7)(29)
- **COORDINATE:** Treatment program referral, if indicated
- **COORDINATE:** Obstetrician referral, if indicated(8)
- **COORDINATE:** Specialist referral for comorbidities, if indicated
- **COORDINATE:** Care team meeting, if needed(11)(12)
- **COORDINATE:** Communication between care team members and care plan reconciliation(11)(13)(14)
- **COORDINATE:** Follow-up contact to review self-care plan and need for further assistance(15)(16)
- **SEND:** Managing your medicines (Illustrated)  (Spanish)
- **SEND:** Be ready for your doctor visit - Substance-related disorders  (Spanish)
- **SEND:** Be ready with questions for your doctor - Substance-related disorders  (Spanish)

P4. ADHD medication evaluation needed

- **Goal:** ADHD medication evaluation completed and medication regimen initiated, if indicated
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with prescribing provider for evaluation of condition status
 - **EDUCATE:** When and how to get help for worsening ADHD problems
 - **EDUCATE:** Medication management needs common for ADHD
 - **COORDINATE:** Follow-up after provider contact for evaluation of medication changes(15)(16)

P5. ADHD medication management needed

- **Goal:** Medication information understood(8)(36)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for discussion on appropriate medication
 - **ASSIST:** Providing reconciled medication list with all providers. See Medication Reconciliation Tool  SR for more information.
 - **EDUCATE: Purpose:** Importance of understanding what each medication is for, how it should be taken, and what to do if dose is missed
 - Stimulant and nonstimulant medications
 - Prescribed to increase cognition and attentiveness as well as to decrease distractibility, hyperactivity, and behavioral disruptions
 - Optimal dose titration may take 4 to 6 weeks.
 - Consequences of nonadherence (eg, to medication regimen)(37)
 - **EDUCATE:** Medication indications, side effects, and considerations frequently change. See a drug reference for specific information not listed here.
 - **EDUCATE:** Administration techniques:
 - Route (eg, oral, intramuscular (IM), intravenous (IV))
 - With or without foods or liquids (eg, milk)
 - Timing of administration
 - Appropriate measuring device (eg, liquid medications)
 - Proper storage
 - **EDUCATE: Get emergency help:** Adverse or allergic reactions (eg, wheezing, chest tightness, fever, itching, bad cough, blue skin color, hemorrhage, seizures, swelling of face, lips, tongue, or throat)
 - **EDUCATE: Side effects:** Management, and when to contact provider
 - Common side effects include gastrointestinal changes, dry mouth, fatigue, insomnia, rash, dizziness, and headache.
 - **EDUCATE: Considerations:**
 - Discuss medication with provider if pregnant, planning a pregnancy, or breast-feeding.
 - Comorbidities may be contraindications to the use of a drug (eg, liver disease, diabetes).
 - Health checks, diary, and action plans (eg, blood pressure, weight, heart rate)
 - Safety measures (eg, sun protection, driving, or operating machinery)
 - Contact provider or pharmacist prior to taking any other over-the-counter or prescribed medication.
 - **COORDINATE:** Clinical monitoring appropriate to medication use (eg, laboratory tests)

- **COORDINATE:** Communication between patient and pharmacy (for obtaining prescriptions, setting up home medication delivery, review for drug-drug interactions)^[N]
- **COORDINATE:** Communication between care team members and medication list reconciliation
- **COORDINATE:** Follow-up to evaluate adherence or problems with medication, provide additional education and support, and identify need for further assistance⁽¹⁵⁾⁽¹⁶⁾
- **COORDINATE:** Laboratory testing, if indicated
- **COORDINATE:** Outpatient imaging, tests, or procedures, if indicated
- **COORDINATE:** Therapeutic drug level monitoring
- **SEND:** Managing your medicines (Illustrated)  (Spanish)

P6. Atomoxetine medication management needed

- **Goal:** Atomoxetine initiated and maintained⁽³¹⁾⁽³⁸⁾⁽³⁹⁾
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** Atomoxetine is a nonstimulant medication used to reduce inattention, hyperactivity, and impulsivity.^[H]
 - First-line treatment with substance use comorbidities
 - Therapeutic response may take 12 weeks.
 - **EDUCATE: Side effects:** Somnolence, hypertension, nausea, abdominal pain, anorexia
 - **EDUCATE: Considerations:** Avoid in patients with glaucoma.

P7. Stimulant medication management needed

- **Goal:** Stimulant medication initiated and maintained⁽³⁵⁾
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** Stimulants may be indicated to reduce inattention, hyperactivity, and impulsivity.⁽³⁸⁾
 - Sustained-release and long-acting formulations available for dose simplification and increased adherence
 - Drug holidays only under supervision of prescribing provider if growth suppression is a concern
 - **EDUCATE: Side effects:** Anxiety, anorexia, hypertension, mood changes, psychotic symptoms, aggressive behavior, tremors, insomnia, drug-seeking behavior⁽⁷⁾
 - **EDUCATE: Considerations⁽⁷⁾:**
 - Take after meals.
 - Take dose early in the day to help avoid insomnia at night.
 - Avoid large amounts of caffeine to prevent increase in restlessness and stimulation.
 - Avoid medications containing stimulants such as caffeine or pseudoephedrine.
 - Stimulants are controlled drugs and should be secured to prevent inappropriate use. Diversion of medications is illegal.
 - Regular monitoring for medication adherence and limiting the risk of stimulant diversion or misuse
 - Use cautiously if psychosis present.⁽⁴⁰⁾
 - **SEND:** Help with sleeping or insomnia  (Spanish)

P8. Healthy diet information needed

- **Goal:** Healthy diet better understood⁽⁴¹⁾⁽⁴²⁾^[N]
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for dietary review and self-care plan
 - **EDUCATE:** Healthy body weight and body mass index (BMI) for patient⁽⁴³⁾:
 - BMI equals body weight in kilograms divided by the square of the height in meters (kg/m²).
 - BMI of 18.5 to 24.9 is a normal weight.
 - BMI of 25 to 29.9 is overweight.
 - BMI of 30 or greater is obese.
 - **EDUCATE:** Healthy diet general recommendations⁽⁴⁴⁾⁽⁴⁵⁾:
 - Well-balanced diet (ie, high in fruits and vegetables, low in sodium, fats, and sugars)
 - Drink enough fluids.
 - Safe eating habits (eg, do not skip meals)
 - Calorie amount appropriate (eg, factors: age, gender, activity level)
 - **EDUCATE:** Adequate fluid intake^{[I](45)(46)}:
 - Amount of fluid intake needed depends on patient attributes, including medical condition, level of physical activity, and exposure to heat.^[N]

- Men age 19 years and older: 15.6 cups (3.7 L) per day from all sources (eg, food and fluids)
- Women age 19 years and older: 11.4 cups (2.7 L) per day from all sources (eg, food and fluids)
- **COORDINATE:** Dietitian referral, if indicated(47)
- **COORDINATE:** Community services referral, as needed
- **COORDINATE:** Follow-up contact with patient to evaluate diet understanding and adherence, referral status, and need for further assistance(15)(16)
- **SEND:** Healthy eating (Illustrated)  (Spanish)
- **SEND:** How to read food labels (Illustrated)  (Spanish)
- **SEND:** Weight diary (Illustrated)  (Spanish)
- **SEND:** Meal diary (Illustrated)  (Spanish)

P9. Potential nutritional problems

- **Goal:** Nutritional problems resolved(42)(48)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for dietary review and self-care plan
 - **ASSIST:** Creation of nutrition diary
 - **COORDINATE:** Laboratory testing, if indicated
 - **COORDINATE:** Calorie or fluid counting
 - **EDUCATE:** Nutritional recommendations for patient's health status(45)
 - **EDUCATE:** Food and drug interactions potentially impacting nutritional status
 - **COORDINATE:** Dietitian referral, if indicated(47)
 - **COORDINATE:** Home care referral, if indicated
 - **COORDINATE:** Pharmacy medication support, if indicated
 - **COORDINATE:** Rehabilitation therapy referral, if indicated
 - **COORDINATE:** Follow-up contact with patient to evaluate nutritional status, referral status, and need for further assistance(15)(16)
 - **SEND:** Eating to keep up your energy  (Spanish)
 - **SEND:** Eating when your stomach is upset  (Spanish)
 - **SEND:** Mouth sores or infections  (Spanish)
 - **SEND:** How to read food labels (Illustrated)  (Spanish)
 - **SEND:** Meal diary (Illustrated)  (Spanish)

P10. Food label reading information needed

- **Goal:** Food label understanding improved
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider, nurse, or dietitian to provide in-person instruction on how to read food labels
 - **EDUCATE:** Food labels contain the nutritional value of the product(49)(50):
 - Percent of daily values is usually based on a 2000-calorie (8374-kJ) diet.
 - Nutritional values are listed per one serving.
 - A package may have more than one serving in the whole package.
 - Nutritional value should be doubled, tripled, and so on based on the number of servings eaten.
 - Serving size varies from product to product.
 - Form of food has impact on nutrition (eg, increased sugar content of orange juice vs one orange).
 - **COORDINATE:** Dietitian referral, if indicated
 - **COORDINATE:** Follow-up contact to evaluate food label understanding, referral status, and need for further assistance(15)(16)
 - **SEND:** How to read food labels (Illustrated)  (Spanish)

P11. Vaccinations not up to date, increasing infection risk

- **Goal:** Vaccinations up to date(51)(52)
 - **Interventions:**

- **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
- **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
- **ASSIST:** Appointment scheduling
- **ASSIST:** Identifying vaccines patient may need that are not contraindicated based on clinical status
- **EDUCATE:** Purpose and benefits of vaccinations
- **EDUCATE:** Recommended immunizations and timing of administration:
 - Centers for Disease Control and Prevention's vaccination schedule: www.cdc.gov/vaccines(53)
 - Vaccine schedules may need to be altered based on medications; check with the specialist provider for information.(54)
 - Possible side effects or complications associated with getting vaccinated
- **EDUCATE:** Influenza vaccination annually(55):
 - Side effects are mild and last 1 or 2 days.
 - Common side effects with shot are sore, red, or swollen area where shot was given; fever; and aches.
 - Common side effects with nasal spray are mild and last 1 or 2 days; they include runny nose, headache, sore throat, and cough.
 - Allergic reaction rare and is usually caused by a chicken egg allergy
- **EDUCATE:** Pneumococcal vaccination potential side effects:
 - Side effects are mild and last 1 or 2 days.
 - Common side effects include a sore, red, or swollen area where shot was given; fever; and muscle aches.
 - Serious side effects are rare.
- **EDUCATE:** COVID-19 vaccination potential side effects(53):
 - Side effects are mild and last 1 or 2 days.
 - Common side effects include a sore, red, or swollen area where shot was given; chills; fatigue; and aches.
 - Serious side effects are rare.
- **COORDINATE:** Community services referral, as needed
- **COORDINATE:** Vaccinations
- **COORDINATE:** Follow-up to evaluate vaccination understanding, referral status, and need for further assistance(15)(16)
- **SEND:** Coronavirus disease 2019 (COVID-19) vaccines 📄 (Spanish)
- **SEND:** Flu vaccine 📄 (Spanish)
- **SEND:** Pneumonia vaccine for adults 📄 (Spanish)
- **SEND:** Hepatitis A vaccine 📄 (Spanish)
- **SEND:** Hepatitis B vaccine 📄 (Spanish)

P12. Social support needed

- **Goal:** Social support improved(56)(57) 📄
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Patient in establishment of a helpful social network(58):
 - Identification of individuals in support network
 - Engaging in social support system (gathering information, helping patient develop skills, encouraging shifts in perspective, and allowing patient to vent about issues)
 - **EDUCATE:** Ways to access social support
 - **EDUCATE:** Role of social support in quality of life (eg, improved self-image, coping)(57)
 - **EDUCATE:** Patient's role in social support relationships (eg, be aware of how actions impact the feelings of family and friends)(59)
 - **COORDINATE:** Behavioral health services referral, if indicated
 - **COORDINATE:** Community services referral, as needed(60)
 - **COORDINATE:** Social worker referral, as needed(10)
 - **COORDINATE:** Support group referral, as needed(61)(62)
 - **COORDINATE:** Support network contact (eg, church, social groups), as needed(58)
 - **COORDINATE:** Follow-up to evaluate social support, referral status, and need for further assistance(15)(16)

Evidence Summary

Quality standards: The Case Management Society of America's Standards of Practice for Case Management principles include facilitation of informed decision making, counseling, and health education with patient self-care that is responsive to the patient's

preferences (eg, culture, values, needs).(63)

Quality standards: Standards indicate that program content should include an individualized self-care plan that includes patient preferences, prioritized goals, interventions with information on whom to contact in urgent situations, and scheduled follow-up. Programs should facilitate communication and collaboration with practitioners in a timely manner.(63)(64)(65)(66)

National guidelines: The 2018 Physical Activity Guidelines Advisory Committee advises that adults with chronic conditions or disabilities, who are able, should follow the key activity guidelines for adults. The guidelines recommend that adults should do at least 150 to 300 minutes a week of moderate-intensity or 75 to 150 minutes a week of vigorous-intensity aerobic physical activity, or an equivalent combination of moderate-intensity and vigorous-intensity aerobic activity. They should also do muscle-strengthening activities on 2 or more days a week. Older adults should do multicomponent physical activity that includes balance training as well as aerobic and muscle-strengthening activities.(67)

Quality standards: Standards indicate that programs should ensure that during care transitions the care plan is re-evaluated, modified, and communicated to appropriate care team members (eg, providers, caregivers) as needed to support the care transition.(63)(64)(65)(66)

Transition consideration: Several studies demonstrated a reduction in 30-day hospital readmissions when the patient follow-up was within 7 days of hospital discharge.(68)(69)(70) Earlier follow-up (eg, within 48 hours after hospital discharge) may also help reduce hospital readmissions for high-risk or vulnerable patients.(15) Follow-up appointment attendance rates were higher when appointments were scheduled before hospital discharge.(71)(72)

Outcomes: Several studies support the role of the pharmacist in medication reconciliation to identify and resolve pharmacy-related problems and improve outcomes (eg, prevention of medication-related adverse events, discontinuation of inappropriate medication).(73)

National guidelines: The Institute of Medicine does not specify how much total fluid should come from water. Their recommendation is based on the median of total water intake on dietary surveys, which include water from liquids and solids. For healthy people, the advice is to consume at least 8 glasses (8 ounces (240 mL) each) of water daily; there is little evidence to support this recommendation.(74)

Quality standards: Standards indicate that program content should address patient supports and psychosocial issues that may impede self-management. Programs should have a process for following up with patients on the status of referrals.(63)

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Footnotes

[A] Assess and address the emergency or urgent problems before continuing the engagement. [A in Context Link 1]

[B] A self-care plan tells you how to take care of your condition and when and where to get help for problems. [B in Context Link 1]

[C] Nongenetic factors (eg, maternal smoking, prematurity, exposure to environmental lead or polychlorinated biphenyls) may also affect the pathogenesis of ADHD.(8)(75) [C in Context Link 1]

[D] Adults with ADHD may sustain an inner sense of restlessness. Attention span tends to increase with age, but still falls behind those who do not have the disorder.(7) [D in Context Link 1]

[E] Psychoeducational therapy focuses on the patient's understanding and awareness of their disorder.(29) Psychosocial therapy includes cognitive behavioral therapy (CBT), coaching, and counseling.(30) CBT provides techniques to change maladaptive cognitive and behavioral patterns.(29) It provides help in acquiring new skills. Young adults in college may benefit from coping skills, problem solving, organizational strategies, and distraction management.(29) [E in Context Link 1]

[F] Telehealth may improve access to and coordination of mental health care. Patients with behavioral health diagnoses may use telehealth or telepsychiatry to access mental health support or care (eg, to get assistance with problem-solving techniques or ask about presenting symptoms or medication use).(76) [F in Context Link 1]

[G] The timing of provider follow-up depends on the patient's disease severity, self-care complexity, patient/caregiver willingness and ability to provide patient care, and available social supports.(77) [G in Context Link 1]

[H] Nonstimulant medication (eg, atomoxetine) is an effective alternative with less potential for misuse.(38) [H in Context Link 1]

[I] Adequate intake is used when the recommended daily allowance cannot be determined because of insufficient data on requirements.(45) [I in Context Link 1]

Codes

ICD-10 Diagnosis: F90.0, F90.1, F90.2, F90.8, F90.9, R41.840

DSM-5: F90.0, F90.1, F90.2, F90.8, F90.9

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